

Adjuvant Endocrine Therapy for Women With Hormone Receptor–Positive Breast Cancer: ASCO Clinical Practice Guideline Focused Update

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The ASCO Clinical Practice Guideline on Adjuvant Endocrine Therapy for Women with Hormone Receptor–Positive Breast Cancer was most recently updated and published in January 2016.¹ ASCO guidelines are updated at regular intervals; however, there may be new evidence that potentially changes a recommendation and becomes available between scheduled updates. ASCO has produced a 2018 focused update in response to new peer-reviewed publications of six randomized clinical trials on extending aromatase inhibitor treatment published since the literature search date cutoff for the 2014 update.²

Focused updates for Clinical Practice Guidelines are approved by the Clinical Practice Guidelines Committee, and this update reflects new evidence in regards to the recommendation on duration of adjuvant endocrine treatment in previous versions of this

guideline. The latter focused on the first 5 years of treatment³ or the duration of therapy for women who receive tamoxifen monotherapy.⁴ The new update summarizes an updated literature search and reviews and analyzes new data in regards to this recommendation reported since the systematic review for the previous update in 2016. The new update has a specific focus on duration of endocrine therapy for postmenopausal women who may have aromatase inhibitor (AI) treatment as part of their initial adjuvant regimen and AI treatment as extended adjuvant endocrine therapy.

The 2018 update does not address the other clinical questions posed in the 2010 guideline or the 2014 and 2016 updates. Additional information is available at www.asco.org/breast-cancer-guidelines. Patient information is available at www.cancer.net.

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AUTHORS' DISCLOSURES OF POTENTIAL CONFLICTS OF INTEREST AND DATA AVAILABILITY STATEMENT

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THE BOTTOM LINE

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Guideline Question Does extended adjuvant therapy, including AIs after 5 years of sequential endocrine therapy improve clinically meaningful outcomes (eg, disease-free survival, overall survival, quality of life, and toxicity) in postmenopausal women with hormone receptor–positive early breast cancer?

Target Population Postmenopausal women with stage I to III hormone receptor–positive breast cancer.

Target Audience Medical, surgical, and radiation oncologists; oncology nurses and physician assistants; general practitioners; and women with stage I to III hormone receptor–positive breast cancer.

Methods An Expert Panel was convened to update clinical practice guideline recommendations based on a systematic review of the medical literature.

Focused Update Recommendations

Recommendation 1. Many women with node-negative breast cancer are potential candidates for and may be offered extended therapy for up to a total of 10 years of adjuvant endocrine treatment, based on considerations of recurrence risk using established prognostic factors. However, because the recurrence risk is lower, the benefits are likely narrower for such patients. Women with low-risk node-negative tumors should not routinely be offered extended therapy.

Recommendation 2. Women with node-positive breast cancer should be offered extended AI therapy for up to a total of 10 years of adjuvant endocrine treatment.

Recommendation 3. Women receiving extended adjuvant endocrine therapy should receive no more than 10 years of total treatment.

Recommendation 4. As prevention of secondary or contralateral breast cancers is a major benefit of extended AI therapy, the risk of second breast cancers (or not) based on prior therapy should inform the decision to pursue extended treatment.

Recommendation 5. Extended therapy carries ongoing risks and side effects, which should be weighed against the potential absolute benefits of longer treatment, in a shared decision-making process between the clinical team and the patient.

Qualifying Statement. To date, none of the studies has shown improvement in overall survival with longer-duration AI therapy. As such, the recommendations on extended adjuvant AI therapy are based on benefits that include prevention of distant recurrence and prevention of second breast cancers.

Additional Resources:

More information, including a Data Supplement with additional evidence tables, a Methodology Supplement with information about evidence quality and strength of recommendations, slide sets, and clinical tools and resources, is available at www.asco.org/breast-cancer-guidelines. Patient information is available at www.cancer.net.

ASCO believes that cancer clinical trials are vital to inform medical decisions and improve cancer care, and that all patients should have the opportunity to participate.

REFERENCES

1. Burstein HJ, Lacchetti C, Anderson H, et al: Adjuvant endocrine therapy for women with hormone receptor-positive breast cancer: American Society of Clinical Oncology clinical practice guideline update on ovarian suppression. *J Clin Oncol* 34:1689-701, 2016
2. Burstein HJ, Lacchetti C, Anderson H, et al: Adjuvant endocrine therapy for women with hormone receptor-positive breast cancer: ASCO clinical practice guideline focused update. *J Clin Oncol*. doi: [10.1200/JCO.18.01160](https://doi.org/10.1200/JCO.18.01160)
3. Burstein HJ, Prestrud AA, Seidenfeld J, et al: American Society of Clinical Oncology clinical practice guideline: Update on adjuvant endocrine therapy for women with hormone receptor-positive breast cancer. *J Clin Oncol* 28:3784-3796, 2010
4. Burstein HJ, Temin S, Anderson H, et al: Adjuvant endocrine therapy for women with hormone receptor-positive breast cancer: American Society of Clinical Oncology clinical practice guideline focused update. *J Clin Oncol* 32:2255-2269, 2014



AUTHORS' DISCLOSURES OF POTENTIAL CONFLICTS OF INTEREST

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